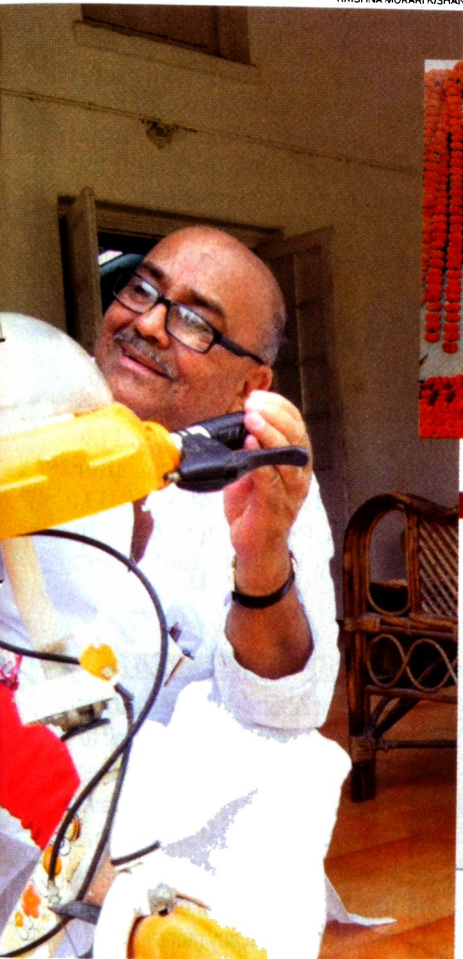


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ABHAY SINGH, 33

On February 26 this year, Abhay, a Janata Dal (United) MLA, killed his wife Sony and an infant daughter Rashi before ending his life with a gun. Three-year-old Khushi, his eldest daughter, survived since she was sleeping with her grandmother in another room. Increasing confrontations with his wife and the pressure of a political career drove Abhay to commit suicide.

KHUSHI WITH HER GRANDFATHER (LEFT); ABHAY AND SONY (ABOVE)



“ Family suicide pacts are a social problem. People often think it is a personal problem and refuse to get involved. ”

HETUKAR JHA, Sociologist

Crime Record Bureau (NCRB) latest report, “Incidence and Rate of Suicide, 2008”, says that the state accounted for 102 of the 290 such cases reported countrywide. Family suicides are being categorised and chronicled since 2001 but it was only in 2008 that Bihar first provided details, going straight to the top of the death diagram. The NCRB report says that while Bihar tops in family suicides, the overall rate of suicide there (the number of suicides per one lakh population) is the second lowest in India at 1.1 against the national average of 10.8. Simply put, individuals are less likely

to commit suicide in Bihar, but families are not. “This is a huge problem requiring urgent intervention,” says sociologist Hetukar Jha. The figures are skewed towards more minor girls than boys dying in family suicides: 44 minor girls and three minor boys.

When families decide to die together, psychiatrists and sociologists consider it a breakdown of community structures. In a place like Bihar, where growing conflicts and the lack of institutional support mechanisms are said to be pushing families into committing suicide, hardly anything is done to study the cause and effects. “It is time

to realise that family suicide pacts are a social problem. People often think it is a personal problem. In fact, it is a manifestation of the social problems which is why the authorities must recognise it,” says Jha.

The NCRB factsheet also negates the widespread impression that family suicides are a feature of urban India and the middle class alone. In 2008, only eight cases totalling 28 victims were registered in the top 35 cities. The remaining were reported from rural and semi-urban India. Bihar clearly appears to be at the crossroads with conflicting signals emanating from a society under transition. There is a sophisticated class with both the potential and opportunity to grow, as well as a much larger traditional group with a lower socio-economic profile. Jha says, “The suicide scenario has to be seen in conjunction with this complex background of social changes.”

The absence of any institutionalised society-family interface only aggravates matters as those who suffer have nobody to turn to. Sociologists and psychologists, however, refute traditional explanations of individualism gaining superiority over families as a cause of suicide. “On the contrary, the desire to kill everyone also underlines strong family ties that seek togetherness in death. This is also done as a compassionate gesture to save children from suffering after their parents’ death,” explains Jha.

Since family relationships are held in high regard, problems originating within the family can be the most difficult to solve, says Dr K.P. Sharma, consultant psychiatrist at the Bihar Institute of Mental Health and Allied Sciences. “The psychological dislocation that causes one to kill oneself has deep social roots,” he says. Because of the great emphasis on family integration in Bihar’s culture, family conflict gains pre-eminence over individual issues. And such conflicts within the family appear to affect traditional societies like Bihar more adversely than other factors, such as depression or poverty, which too contribute to an extent. “Family problems are factors for